# 5.18 Delirium

| Assessment | Nursing Diagnosis | Plan / Expected Outcome | Nursing Interventions | Evaluation |
|---|---|---|---|---|
| **Subjective**<br>• Family say<br>• "He is not himself since yesterday."<br>**Objective**<br>• Attention comes and goes<br>• Disoriented<br>• Agitated at night | **ACTUAL**<br>**Acute confusion** related to (cause, for example infection, drugs, dehydration) as evidenced by fluctuating consciousness and disorientation. | Back to his usual orientation as the cause is treated. | • Find and treat the cause together with the medical team.<br>• Reorient him often; keep his glasses and hearing aids on.<br>• Encourage family to be with him.<br>• Help him sleep at night and stay active in the day.<br>• Avoid restraints and any sedative that is not needed. | Oriented: **goal met**. |
| **Subjective**<br>• Family: "He sleeps all day and is awake at night."<br>**Objective**<br>• Awake at night<br>• Sleeps in the day | **ACTUAL**<br>**Disturbed sleep pattern** related to confusion as evidenced by night waking. | Within 3 days he sleeps at night and is awake in the day. | • Keep noise and light down at night.<br>• Open the curtains and keep him active in the day.<br>• Do not wake him at night unless you need to. | Sleeping better: **goal met**. |
| **Subjective**<br>• *No specific complaint*<br>**Objective**<br>• Agitated<br>• Keeps trying to climb out of bed | **RISK**<br>**Risk for adult falls** as evidenced by risk factors: confusion and agitation. | No falls. | • Fall precautions in place.<br>• Keep him under close observation.<br>• Keep the call bell and anything he needs within reach. | No falls: **goal met**. |